

PRODUCT LITERATURE

BUGESIC ORAL SUSPENSION 100 MG/ 5 ML

Each 5 ml contains:

Ibuprofen	100mg
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Preservatives:

Methyl Paraben	0.1%w/v
Propyl Paraben	0.01%w/v

Description

Pink coloured thick suspension with sweet taste and strawberry odour.

Pharmacodynamics properties

Ibuprofen is a Phenylpropionic Acid derivative, which has analgesic, anti-inflammatory and antipyretic actions. These actions are thought to be from its inhibitory effect on the enzyme cyclo-oxygenase which results in a reduction/inhibition of prostaglandin synthesis.

Pharmacokinetics properties

Ibuprofen is rapidly absorbed from the gastro-intestinal tract and rapidly distributed throughout the whole body. Peak plasma concentrations occur about 1 to 2 hours after ingestion with food or in 45 minutes if taken on an empty stomach. These times may vary with different dosage forms.

The excretion is rapid and complete via the kidneys.

The elimination half-life is about 2 hours.

It is metabolized to two inactive metabolites and these are rapidly excreted in urine.

About 1 percent is excreted in urine as unchanged Ibuprofen and about 14 percent as conjugated Ibuprofen.

Ibuprofen is extensively bound to plasma proteins.

In limited studies, Ibuprofen appears in breast milk in very low concentrations.

Indication

Bugesic is indicated for its anti-inflammatory and analgesic effects in the treatment of rheumatoid arthritis, osteoarthritis and non-articular rheumatism. It is also indicated for its analgesic effect in the relief of mild to moderate pain such as dysmenorrhoea. Its antipyretic activity is effective in reducing fever.

Route of administration

Oral.

Recommended Dose

To be taken 3 or 4 times a day after meals:

Children:

Not recommended for children weighing less than 7kg.

The daily dosage of BUGESIC is 20 mg/kg of bodyweight in divided doses.

This can be achieved as follows:

1 - 2 years: One 2.5 ml spoonful (50 mg)

3 - 7 years: One 5 ml spoonful (100 mg)

8 - 12 years: Two 5 ml spoonful (200 mg)

In juvenile rheumatoid arthritis, up to 40 mg/kg of bodyweight daily in divided doses may be taken.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Contraindication

Hypersensitivity to Ibuprofen or any of the other constituents. Patients who have previously shown hypersensitivity reactions (e.g. asthma, rhinitis, angioedema or urticaria) in response to aspirin or other non-steroidal anti-inflammatory drugs.

Active or previous peptic ulcer. History of upper gastrointestinal bleeding or perforation, related to previous NSAIDs therapy. Use with concomitant NSAIDs including cyclo-oxygenase-2 specific inhibitors (see section, Interaction). Severe hepatic failure, renal failure or heart failure (see section special warnings). Last trimester of pregnancy (see section, Pregnancy)

Warning and Precautions

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problem (eg dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GIT symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (eg alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seems to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Cautions (discussion with doctor or pharmacist) is required prior to starting treatment in patients with a history of hypertension and/or heart failure as fluid retention; hypertension and oedema have been reported in association with NSAID therapy.

The elderly are at an increased risk of serious consequences of adverse reactions. Systemic lupus erythematosus and mixed connective tissue disease - increased risk of aseptic meningitis.

Chronic inflammatory intestinal disease (ulcerative colitis, Crohn's disease) - as these conditions may be exacerbated.

Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms.

Cardiovascular and cerebrovascular effects

Clinical trial and epidemiological data suggest that use of ibuprofen, particularly at high doses (2400mg daily) and in long-term treatment may be associated with a small increased risk of arterial thrombotic events (for example mycardial infarction or stroke). Overall, epidemiological studies do not suggest that low dose ibuprofen (e.g. 1200mg daily) is associated with an increased risk of myocardial infarction.

There is some evidence that drugs which inhibit cyclooxygenase / prostaglandin synthesis may cause impairment of female fertility by an effect on ovulation. This is reversible on withdrawal of treatment. The use of Ibuprofen is therefore not recommended in women attempting to conceive.

GI bleeding, ulceration or perfusion, which can be fatal, has been reported with all NSAIDs at any time during treatment, with or without warning symptoms or a previous history of serious GI events.

Patients with a history of GI toxicity, particularly when elderly, should report any unusual abdominal symptoms (especially GI bleeding) particularly in the initial stages of treatment.

Caution should be advised in patients receiving concomitant medications which could increase the risk of gastrotoxicity or bleeding, such as corticosteroids, or anticoagulants such as warfarin or anti-platelet agents such as aspirin.

Where GI bleeding or ulceration occurs in patients receiving Ibuprofen, the treatment should be withdrawn.

Administration of NSAIDs such as Ibuprofen may cause dose dependent renal toxicity in patients with reduced renal blood flow or blood volume where renal prostaglandins support the maintenance of renal perfusion.

Patients at risk of this reaction include those with impaired renal function, heart failure or liver dysfunction. This is of particular importance in hypertension and/or cardiac impairment as renal function may deteriorate and/or fluid retention occurs. Caution is therefore required in the use of Ibuprofen in such patients.

Ibuprofen should be used with caution in patients with bronchial asthma or allergic disease, since such patients may have NSAID-sensitive asthma which has been associated with severe bronchospasm.

Cardiovascular Thrombotic Events

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.

There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure

Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity.

The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Do not give this product if your baby or child

-Has or has ever had stomach ulcer, perforation or bleeding

-Is allergic to Ibuprofen or any other ingredient of the product, aspirin or other related painkillers

-Is taking other NSAID painkillers, or aspirin with a daily dose above 75mg
Speak to a pharmacist or your doctor before giving this product if your baby or child
-Has asthma, liver, heart, kidney or bowel problems
If you are an adult taking this product you should not take Ibuprofen in the last 3 months of pregnancy and you should contact your doctor or pharmacist before taking it in the first 6 months of pregnancy or if trying to get pregnant.
Do not give to babies aged 3-6 months for more than 24 hours.
Do not give to children aged 6 months and older for more than 3 days.
If symptoms persist or worsen, consult your doctor promptly.
Do not exceed the stated dose.
Not recommended for children under 3 months.

Interactions with Other Medicaments

Ibuprofen should not be used in combination with:

Aspirin: Unless low-dose aspirin (not above 75mg daily) has been advised by a doctor, as this may increase the risk of adverse reactions.

Other NSAIDs: As these may increase the risk of adverse effects.

Ibuprofen should be used with caution in combination with:

Anti-coagulants: NSAIDs may enhance the effects of anti-coagulants, such as warfarin.

Anti-hypertensives and diuretics: NSAIDs may diminish the effect of these drugs.

Corticosteroids: May increase the risk of adverse reactions in the gastrointestinal tract

Lithium: There is evidence for potential increases in plasma levels of lithium.

Methotrexate: There is potential for an increase in plasma methotrexate

Zidovudine: There is evidence of an increased risk of haemarthroses and haematoma in

HIV (+) haemophiliacs receiving concurrent treatment with zidovudine and Ibuprofen.

Pregnancy and Lactation

While no teratogenic effects have been demonstrated in animal experiments, the use of Ibuprofen should, if possible, be avoided during the first 6 months of pregnancy.

During the 3rd trimester, Ibuprofen is contraindicated as there is a risk of premature closure of fetal ductus arteriosus with possible persistent pulmonary hypertension. The onset of labour may be delayed and the duration increased with an increased bleeding tendency in both mother and child.

In limited studies, Ibuprofen appears in breast milk in very low concentration and is unlikely to affect breast-fed infants adversely.

Side Effects

Oedema, hypertension, and cardiac failure, have been reported in association with NSAID treatment.

Hypersensitivity reactions have been reported and these may consist of:

- Non-specific allergic reactions and anaphylaxis,
- Respiratory tract reactivity e.g. asthma, aggravated asthma, bronchospasm or dyspnoea,
- Various skin reactions, e.g. pruritis, urticaria, purpura, angioedema and more rarely exfoliative dermatitis and bullous dermatoses (including epidermal necrolysis and erythema multiforme).

The following list of adverse effects relates to those experienced with Ibuprofen at OT C doses, for short term use. In the treatment of chronic conditions, under long term treatment, additional adverse effects may occur.

Clinical trial and epidemiological data suggest that use of ibuprofen (particularly at high doses 2400mg daily) and in long-term treatment may be associated with a small increased risk of arterial thrombotic events (for example myocardial infarction or stroke)

Hypersensitivity reactions:

Uncommon: Hypersensitivity reactions with urticaria and pruritus.

Very rare: Severe hypersensitivity reactions. Symptoms could be facial, tongue and laryngeal swelling, dyspnoea, tachycardia, hypotension (anaphylaxis, angioedema and severe shock).

Exacerbation of asthma and bronchospasm.

Gastrointestinal:

Uncommon: abdominal pain, nausea and dyspepsia.

Rare: diarrhoea, flatulence, constipation and vomiting.

Very rare: peptic ulcer, perforation or gastrointestinal haemorrhage, sometimes fatal, particularly in the elderly. Exacerbation of ulcerative colitis and Crohn's disease.

Nervous System:

Uncommon: headache

Renal:

Very rare: acute renal failure, papillary necrosis. Especially in long term use, associated with increased serum urea and oedema.

Hepatic:

Very rare: liver disorders

Haematological:

Very rare: haematopoietic disorders (anaemia, leucopenia, thrombocytopenia, pancytopenia, agranulocytosis). First signs are: fever, sore throat, superficial mouth ulcers, flu-like symptoms, severe exhaustion, unexplained bleeding and bruising.

Skin:

Uncommon: various skin rashes

Very rare: severe forms of skin reactions such as erythema multiforme and epidermal necrosis can occur.

Immune System:

In patients with existing auto-immune disorders (such as systemic lupus erythematosus, mixed connective tissue disease) during treatment with ibuprofen, single cases of symptoms of aseptic meningitis, such as stiff neck, headache, nausea, vomiting, fever or disorientation have been observed.

Symptoms and Treatment of Overdose

In children, ingestion of more than 400mg/kg may cause symptoms. In adults the dose response effect is less clear cut. The half-life at overdose is 1.5-3 hours.

Symptoms

Most patients who have ingested clinically important amounts of NSAIDs will develop no more than nausea, vomiting, epigastric pain, or more rarely diarrhoea. Tinnitus, headache and gastrointestinal bleeding are also possible. In more serious poisoning, toxicity is seen in the central nervous system, manifesting as drowsiness, occasionally excitation and disorientation or coma. Occasionally patients develop convulsions. In serious poisoning metabolic acidosis may occur and the prothrombin time/ INR may be prolonged, probably due to interference with the actions of circulating clotting factors. Acute renal failure and liver damage may occur. Exacerbation of asthma is possible in asthmatics.

Management

Management should be symptomatic and supportive and include the maintenance of a clear airway and monitoring of cardiac and vital signs until stable. Consider oral administration of activated charcoal if the patient presents within 1 hour of ingestion of a potentially toxic amount. If frequent or prolonged, convulsions should be treated with intravenous diazepam or lorazepam. Give bronchodilators for asthma.

Packing

Packed in plastic container of 60 ml and 100 ml.

Storage Conditions

Store below 30°C in a dry place, protected from direct light.

Keep away from reach of children.

Jauhkan daripada kanak-kanak

SHAKE WELL BEFORE USE

Expiry Date

2 years from date of manufacture

Name and Address of Product Registration Holder/ Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)

Lot 10 & 11, PERDA Industrial Park,

Lorong IKS Simpang Ampat B,

14100 Simpang Ampat, S.P.S.,

Pulau Pinang, Malaysia

Name and Address of Manufacturer:

TERAPUTICS SDN. BHD. (590500W)

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